



GOVERNMENT OF THE PUNJAB • SPECIALIZED HEALTHCARE & MEDICAL EDUCATION

## DEPARTMENT OF DIAGNOSTIC RADIOLOGY

GUJRANWALA TEACHING HOSPITAL | GUJRANWALA MEDICAL COLLEGE

**HOD:** Prof. Dr. Mian Waheed Ahmad • **Senior Registrars:** Dr. Samiya Shahzadi / Dr. Sidra Ghaffar



Patient Name **Farwa**

Token / Radiology ID **#7057**

Age / Gender **37 Years / Female**

Date of Exam **03-Sep-2026**

Modality & Protocol **High-Resolution Computed Tomography (HRCT) of the Chest**

Reporting Date 03-Sep-2026

Referring Clinician Pulmonology OPD

Clinical History: Chronic dry cough and mild exertional dyspnea. Evaluation for interstitial lung disease.

Comparison: No previous imaging available for comparison.

Technique

High-resolution volumetric thin-collimation (1.0 mm) non-contrast CT of the chest was performed in full inspiration with lung and mediastinal reformations.

Findings Organ-By-Organ System

## Tracheobronchial Tree & Airways

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**Trachea & Central Bronchi:** The trachea and main bronchi are normal in caliber and patent with no intraluminal lesion or tracheomalacia.

## Pulmonary Parenchyma

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**Right Middle Lobe:** Focal mild interlobular septal thickening is noted in the right middle lobe without consolidation or cavitation, likely representing post-inflammatory change.

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**Bilateral Lung Fields:** Remainder of bilateral upper, middle, and lower lobes are clear with no reticulation, honeycombing, traction bronchiectasis, or architectural distortion.

## Pleura & Mediastinum

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**Pleural Spaces:** No pleural effusion or pneumothorax on either side.

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**Mediastinal & Hilar Nodes:** A few subcentimeter precarinal lymph nodes are noted, non-enlarged by CT size criteria, likely reactive.

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**Cardiovascular Structures:** Heart is normal in size. Main pulmonary artery diameter is within normal limits (23.5 mm).

## Impression

1. Focal mild right middle lobe interlobular septal thickening without architectural distortion or honeycombing, likely representing localized post-inflammatory changes.
2. Subcentimeter reactive mediastinal lymph nodes.
3. No CT evidence of usual interstitial pneumonia (UIP) or active pulmonary consolidation.

## Recommendations

Clinical correlation with pulmonary function tests (PFTs) is advised; routine follow-up as clinically indicated.

Primary Reporting Radiologists

**Dr. Sana Fatima** — MBBS (WMO Diagnostic Radiology)

**Dr. Faisal Maqsood Anwar** — MBBS, FCPS-II Resident

**Dr. Areeba** — MBBS, FCPS-II Resident

Reviewed & Approved By (Consultants)

Prof. Dr. Mian Waheed Ahmad

MBBS, FCPS, BSc (Radiology) • Head of Department

Dr. Samiya Shahzadi / Dr. Sidra Ghaffar (Senior Registrars)

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